

THE GLP-1 LIFESTYLE SERIES

Managing GLP-1 Side Effects

A Practical Handbook for the First 90 Days

Disclaimer

This book is for educational and informational purposes only. It is not medical advice and does not constitute a recommendation to start, stop, or change any medication or treatment. GLP-1 receptor agonists are prescription medications that should only be used under the supervision of a qualified healthcare provider.

The nutritional guidance, exercise information, and lifestyle strategies in this book are general wellness information and are not tailored to your individual medical situation. Always consult your doctor, registered dietitian, or other qualified health professional before making changes to your diet, exercise routine, or medication management.

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What to Expect — The First 90 Days Honestly

The first ninety days on GLP-1 medication are often the most challenging. They're also when most people quit — not because the medication isn't working, but because the side effects in the early weeks are genuinely uncomfortable and nobody prepared them adequately for what was coming.

The Side Effect Timeline

GLP-1 side effects are not uniformly distributed across time. They tend to peak in the first two to four weeks of a new dose, then improve as the body adapts. Dose escalations restart the cycle at a lower intensity than the initial start.

The most common side effects and their typical timeline: nausea (first two to six weeks, usually most intense in weeks one and two), fatigue (first two to four weeks), constipation (ongoing, but typically manageable within four to six weeks), headaches (usually first week, then episodic), and acid reflux or heartburn (variable, often improves within a month).

Knowing this timeline matters because it reframes the experience. Week two nausea feels different when you know it's the worst it will get rather than a sign the medication isn't agreeing with you.

The Dose Escalation Protocol

GLP-1 medications use a dose escalation schedule precisely because side effects are dose-dependent. Starting low and increasing slowly allows the body to adapt. The schedules vary by medication — semaglutide (Ozempic/Wegovy) typically escalates monthly, tirzepatide (Mounjaro) every four weeks.

If side effects are severe on a given dose, it is entirely appropriate to remain at that dose for longer than the standard schedule before escalating. Slower escalation is associated with better tolerability without significantly compromising efficacy. Discuss this with your prescribing provider.

"The side effects feel permanent when you're in them. They almost never are."

Nausea — Managing the Most Common Side Effect

Nausea on GLP-1 is the most frequently reported side effect and the most common reason people discontinue. Most of it is preventable or significantly reducible with the right strategies.

Dietary Strategies for Nausea

Small, frequent portions. This is the most important instruction. The stomach is processing food more slowly than usual, and adding volume to an already-processing stomach creates the pressure and discomfort that produces nausea. Aim for portions that fill about a third to a half of what you'd normally eat, eaten three to four times per day.

Avoid high-fat foods during high-nausea periods. Fat is the macronutrient that most significantly slows gastric emptying — it stays in the stomach longest. Fried foods, creamy sauces, fatty meats, and large amounts of added fat compound the delayed emptying already produced by the medication.

Cold and bland foods are generally better tolerated. Many GLP-1 users find that hot food smells and rich

flavours trigger nausea even when they didn't before. Trust these aversions during the adaptation period — your body is giving you useful information about what it can currently process.

Non-Dietary Strategies

Timing the injection strategically can reduce nausea. Many people find that taking the weekly injection at bedtime (for weekly formulations) means the peak nausea period occurs during sleep. Discuss this option with your prescriber.

Ginger is the most evidence-supported natural nausea remedy and is safe alongside GLP-1. Ginger tea, ginger chews, or ginger capsules all provide some benefit. The mechanism involves serotonin receptor modulation in the gut — similar to some prescription anti-nausea medications.

Vitamin B6 (25mg, three times daily) has reasonable evidence for reducing pregnancy-related nausea and is sometimes effective for GLP-1 nausea through similar mechanisms. Check with your doctor before adding supplements.

"Nausea on GLP-1 is mostly about management strategy, not endurance."

Constipation and Digestive Changes

Constipation is the second most common GLP-1 side effect, and unlike nausea, it often persists beyond the adaptation period and requires ongoing management.

Why GLP-1 Causes Constipation

Delayed gastric emptying doesn't just affect the stomach — it slows the entire GI tract. Food moves more slowly through the intestines, allowing more water to be reabsorbed, which produces harder, drier stools. Combined with reduced food intake (less fibre) and often reduced water intake (nausea makes drinking unappealing), constipation on GLP-1 is a predictable consequence rather than an unusual side effect.

The Constipation Management Protocol

Hydration is the most important single intervention — at least 2–2.5 litres of water per day, even when nausea makes drinking unappealing. Sipping steadily throughout the day is more tolerable than large

amounts at once.

Fibre from food: prioritise soluble fibre (oats, beans, lentils, fruits, psyllium) over insoluble fibre in the early stages. Soluble fibre adds bulk and retains water in the stool. Very high insoluble fibre (bran, raw vegetables in large amounts) can worsen bloating when gut motility is already reduced.

Movement: even gentle walking stimulates gut motility. Ten to fifteen minutes of walking after meals is one of the most effective non-pharmaceutical interventions for constipation.

Magnesium citrate or glycinate (300–400mg at bedtime) has both laxative effects and general health benefits, and is well-tolerated for most people. Polyethylene glycol (Miralax in the US) is the over-the-counter osmotic laxative most commonly recommended by gastroenterologists for ongoing management.

*"Constipation on GLP-1 is manageable.
Ignoring it is not – it becomes uncomfortable
quickly and compounds other side effects."*

Fatigue, Headaches, and Other Early Side Effects

The non-gastrointestinal side effects of GLP-1 are less discussed but equally real in the adaptation period.

Fatigue

GLP-1 fatigue in the early weeks is largely driven by calorie deficit (eating less than the body is used to) and the metabolic adjustment of significant appetite suppression. The body is working with less fuel than it's accustomed to. This typically improves within two to four weeks as the body adapts to the new energy intake.

During the fatigue period: prioritise sleep, maintain hydration and protein intake (both reduce fatigue), and reduce the intensity of exercise temporarily if needed. This is not the time to add new training demands. Gentle movement is more appropriate than high-intensity exercise while the body is adapting.

Headaches

Headaches in the first days to weeks are most commonly related to dehydration and low calorie intake

rather than the medication's direct effects. The approach is simple: increase fluid intake, don't skip meals entirely, and ensure adequate sodium intake (which helps retain fluid and maintain blood pressure, both of which can drop slightly with significant calorie reduction).

Acid Reflux and Heartburn

Delayed gastric emptying allows stomach acid more time to contact the lower oesophagus, which can worsen or create acid reflux. Strategies: don't lie down for at least one to two hours after eating, sleep with the head slightly elevated if reflux is nocturnal, avoid known reflux triggers (caffeine, alcohol, fatty foods, citrus), and eat smaller portions.

If reflux is severe or persistent, speak with your doctor about appropriate treatment. Over-the-counter antacids are generally safe with GLP-1, but proton pump inhibitors or H2 blockers may be appropriate if reflux is significant.

"Most early GLP-1 side effects are the body adjusting. Support it through the adjustment."

When to Call Your Doctor

Most GLP-1 side effects are uncomfortable but manageable. Some require medical attention.

Side Effects That Need Medical Evaluation

Severe, persistent vomiting that prevents adequate hydration (risk of dehydration and electrolyte imbalance). Severe abdominal pain, particularly radiating to the back (rare risk of pancreatitis, a known but uncommon GLP-1 side effect). Significant changes in vision (associated with rapid blood sugar changes in people with diabetes). Any symptoms that are rapidly worsening or not improving after the typical adaptation period.

Pancreatitis is the most serious GLP-1 risk. The symptoms — severe upper abdominal pain, often radiating to the back, sometimes with nausea and fever — are distinctive and warrant immediate medical evaluation. The absolute risk is low, but it's important to know the warning signs.

Building a Working Relationship with Your Prescriber

GLP-1 treatment is most effective when managed collaboratively with a knowledgeable provider. If your current prescriber is unfamiliar with GLP-1 side effect management or isn't responsive to your concerns during the adaptation period, a provider with specific obesity medicine or endocrinology training may be worth seeking. The medication is too useful to abandon because of manageable side effects that received inadequate guidance.

*"Discomfort is expected. Distress is a signal.
Know the difference."*
